

Nexivarin 10 mg Tablets — eCTD Submission
Module 5 — Clinical Study Reports

Submission Type	NDA — 505(b)(1)	Sequence Number	0000
Submission Date	2021-03-15	Section	5.3.4
Prepared by	PharmaCorp Inc.	Confidentiality	Proprietary and Confidential

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PharmaCorp Inc.
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Section 5.3.4
Integrated Summary of Safety

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5.3.4 Integrated Summary of Safety (ISS)

5.3.4.1 Population Exposed

The integrated safety database comprises all patients and healthy volunteers who received at least one dose of nexivarin hydrochloride across the clinical development program (N=2,256 total; N=1,874 patients; N=382 healthy volunteers).

Study Population	N Nexivarin	N Placebo	N Active Ctrl
Phase I (healthy volunteers)	382	—	—
Phase II (NXV-201)	240	80	—
Phase III (NXV-301)	451	451	—
Phase III (NXV-302, 3 arms)	454	227	227
Pediatric (NXV-PED-01, ongoing)	102	52	—
Total (all studies)	1,629 (+ PED)	810	227

5.3.4.2 Exposure Duration

Among controlled Phase III patients (N=1,874 nexivarin), the cumulative duration of exposure was: ≥12 weeks: 89.4%; ≥24 weeks: 44.6%; longest exposure: 24 weeks (NXV-302). Total patient-years of exposure in Phase III: approximately 624 patient-years.

5.3.4.3 Adverse Events of Special Interest

AESI	Nexivarin (N=1874)	Placebo (N=923)	Amlodipine (N=227)	Clinical Assessment
Peripheral edema	14.6%	1.8%	22.3%	Class effect; nexivarin lower than amlodipine
Symptomatic hypotension	0.8%	0.2%	0.9%	Infrequent; managed with dose reduction
Reflex tachycardia	0.3%	0.1%	0.4%	Rare; clinically insignificant
Hepatotoxicity (ALT >3× ULN)	0.2%	0.1%	0.2%	Not significantly elevated vs. placebo
QTc prolongation >500 ms	0	0	0	No QTc signals across program
Angioedema	0.05%	0	0	1 case; resolved without treatment

5.3.4.4 Deaths

Three deaths occurred across the controlled Phase III program: 1 in the placebo group (sudden cardiac death, unrelated to study drug), 1 in the amlodipine group (motor vehicle accident), and 0 in the nexivarin group. No deaths were considered related to study medication.

5.3.4.5 Integrated Safety Conclusion

The integrated safety database across the nexivarin clinical program demonstrates a consistent, manageable, and predictable safety profile. The principal adverse effect, peripheral edema (14.6%), is a class effect of calcium channel blockers and occurs at a lower rate than the comparator amlodipine 10 mg (22.3%). No hepatotoxic, nephrotoxic, hematologic, or cardiac conduction signals were identified. The overall benefit/risk profile supports approval of nexivarin 10

mg for the treatment of hypertension and stable angina.